

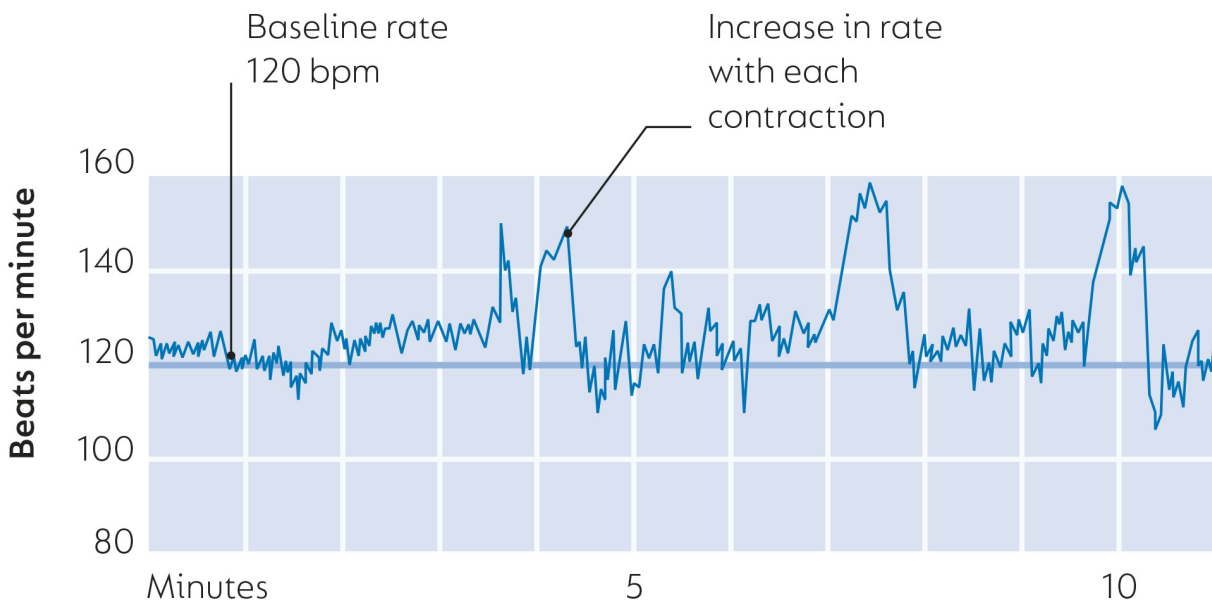
during a vaginal examination. It may be mildly uncomfortable for your baby and there is a small risk that your baby could get a scalp infection, which can be treated with antibiotics after the birth. Even though these risks are small, a scalp electrode should not be placed routinely. Your doctor should discuss how the device works before it is placed, and you should understand why it is being done. Scalp electrodes should not be used if you have a viral disease that could be transmitted to your baby. Once a scalp electrode has been placed, you can't move far from the monitor, although you may be able to change position.

Labor graphs

A labor graph is a large chart that contains several graphs that provide information on your labor, allowing the doctor to monitor the progress of your labor. One of the most useful tools in this chart is a graph showing your labor curve. This plots cervical change and the position of your baby's head in relation to your pelvis over time. The graph enables the doctor to establish when your labor became active. Also recorded with your baby's heart-rate monitoring are your blood pressure, pulse, temperature, and the rate of your contractions, as well as your pain levels.

EXTERNAL ELECTRONIC FETAL MONITORING

The baby's heartbeat and the frequency of your contractions are measured by devices strapped to your abdomen with wires that connect to a machine that produces a printout of the readings and an ongoing digital log on the computer.



Your baby's heartbeat will vary with a normal range of 110–160 beats per minute (bpm). The heart rate rises and falls naturally with contractions; unusual variations can indicate fetal distress.